Nivolumab, ipilimumab, pemetrexed and carboplatin

National Cancer Control Programme (NCCP) 00713 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending

PRACTICAL TREATMENT GUIDE

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Alternating visits and duration

The first cycle lasts 42 days: all four medicines on Day 1, then nivolumab, pemetrexed and carboplatin on Day 22. Chemotherapy ends after those two visits. From Cycle 2, nivolumab continues on Days 1 and 22 and ipilimumab on Day 1 of each 42-day cycle. Immune treatment continues while appropriate until the cancer progresses, side effects require stopping, or up to 24 months without progression. Plans can change after review.

STEP 2

Time in the Day Ward

Nivolumab over 30 minutes first; ipilimumab over 30 minutes on Day 1 only; pemetrexed over 10 minutes; carboplatin over 30 minutes. Ipilimumab requires vital signs every 30 minutes during infusion and for one hour afterwards. Clinic time also includes assessment, preparation, observation and support.

STEP 3

Medicine and monitoring appointments

Folic acid starts before pemetrexed, continues through both chemotherapy visits and for 21 days after the last pemetrexed dose. B12 is given in the preceding week. The source repeats B12 every three cycles if pemetrexed continues; this planned course has only two pemetrexed administrations, so do not automatically continue B12 in immune-only cycles. Your team reconciles the steroid used to prevent pemetrexed rash with the anti-sickness steroid into one clear prescription. Do not add the two schedules together or use leftover steroids yourself.

STEP 4

Transport and access

intravenous (IV) treatment through agreed venous access; no home chemotherapy pump. Arrange transport, lab appointments and prescriptions. Renal testing is needed for the consultant-selected carboplatin AUC 5 or 6. Keep immune alert information available for every healthcare contact.

Sources: NCCP 00713 v4 · official regimen. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.